Planning considerations

A short guide to the full interactive board, images and original scans.

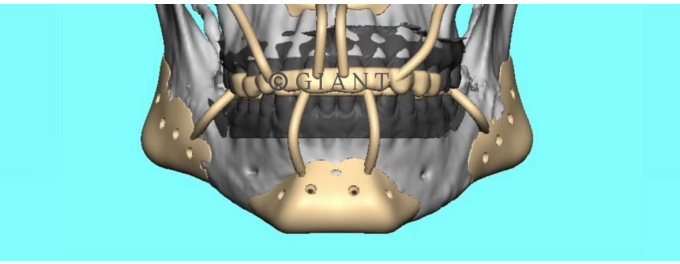
Jaw revision

Preferred: replace both angles. Reduce unwanted bulk, refine both contours and preserve the wanted corner height. Ivan is concerned about retaining the current implants; comparative risks still need clinical assessment.

Design reference: I like Liam's **new jaw-angle design**.

Design reference alternative: retain the left implant and replace only the right with a smoother design and a little more width for symmetry with the misplaced left side.

Prefer an intraoral coverage/repair attempt. External access or fat needs separate discussion.



Liam's NEW yellow/gold proposal - jaw-area crop. Original image on the board; final STL / STEP not supplied.

Localised fat

Consider a **small localised RIGHT nostril-base fat graft** at examination; no amount selected.

Relevant history

December 2021 wraparound; December 2022 current jaw/chin and infraorbitals; July 2023 postoperative CT; July 2026 masseter Botox.

Lift & selective fat

Discussed: transconjunctival midface and small temple lift while retaining infraorbital projection. Cömert described contour improvement as uncertain; shape may remain visible. Assess fixation, selective fat or later replacement.

LEFT lateral orbital line

Visible line reported at the left outer orbital area. Its cause has not been established.

Assess whether the planned lower-eyelid access can reach the line and whether smoothing, tissue coverage or limited filling would help. Ivan wonders about a small bone graft; this is a consideration, not a selected addition. Access and suitability need confirmation.

Upper- and lower-eye morphs are independent appearance illustrations.

Rhinoplasty

Preserve the liked profile and rotation while addressing asymmetry. V3/V4 are disliked references.

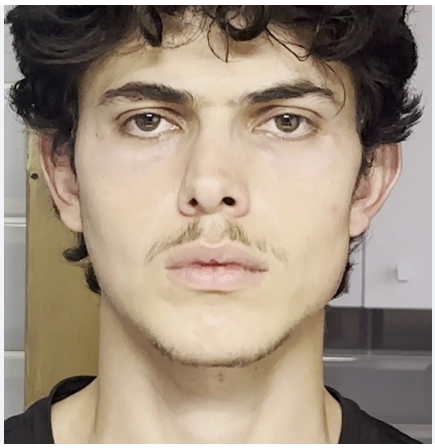
Breathing: improve nasal breathing and address the reported one-sided collapse. Confirm the anatomical side and assess nasal valve, septum and sidewall support. Ivan identifies the affected side as also needing more bridge support despite its overbony appearance.

Open the full interactive board

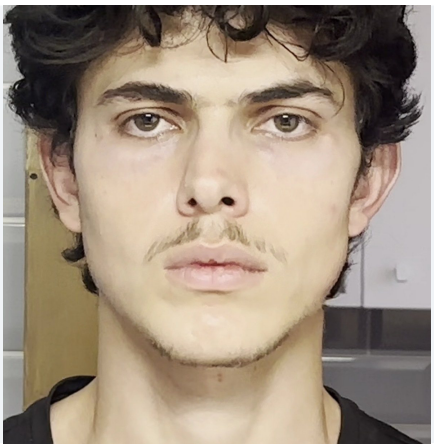
Visual references & files

Photo edits communicate appearance preferences. They are not operative CAD or predicted results.

Jaw / current

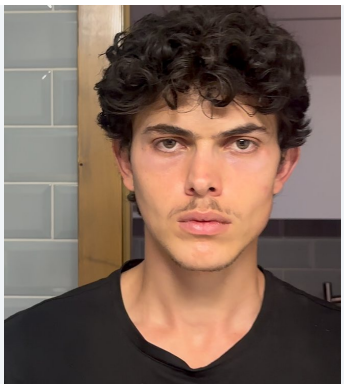


Jaw / B width illustration



B retains A's earlier right-width addition and illustrates left narrowing only. Approximate inputs: right +2 mm above the corner / +1 mm at it; left up to ~2.8 mm inward, with no vertical shift. Final width and contour remain undecided. B does not use Liam's new proposal.

Nose / current



Nose / V1 alignment reference



V1 is one appearance reference, not a selected final nose. Millimetres are uncalibrated. The board includes other views, nostril studies and V3/V4 as disliked references. Static morphs do not assess breathing.

Evidence & full studies

CT surfaces and placement overlays support review; verify registration independently. Segmentation does not establish muscle-sling attachment or named fat-pad descent. 2022 is the pre-infra baseline.

Independent segmentation/VSP remains necessary. Original manufacturer CAD is a separate source; Liam's cleaned jaw scan is optional. Shown millimetres are model inputs unless identified as measurements.

Board: jaw, eyes, nose, skull overlays, masseter and tissue studies

DOWNLOADS

Download board + companion files (ZIP)

Download everything, including all four CT archives

Liam's original 2022 infraorbital report

Original CTs: 2022 pre-infra / 2023 postoperative / 2024 / 2026. All linked in the board.

Surgical access reference: AO Foundation

Breathing reference: AAO-HNS nasal valve statement